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/ **Practice Exam - 1**

Correct Answer

Practice Exam - 1

Question 88 of 117

Next Q



A 78-year-old woman with a history of HFpEF, HTN, DM and gout presented to outpatient clinic with worsening shortness of breath and fatigue. Vitals signs were notable for a blood pressure of 118/84 mmHg, heart rate of 62 bpm and her oxygen saturation was 98% on room air. Physical examination was notable for a JVP of 6 mmHg with a negative HJR, regular rate and rhythm with a soft systolic ejection murmur and no lower extremity edema. Medical regimen included spironolactone 50 mg daily, furosemide 20 mg daily, sacubitril-valsartan 24-26 mg twice daily, dapagliflozin 10 mg daily, metoprolol 150 mg daily, and metformin 500 mg twice daily. Lab work including a CBC, CMP, TSH and iron studies was unrevealing. TTE showed an EF of 72% with a collapsed IVC. Nuclear stress testing was negative.

What is the appropriate next treatment step to help address her symptoms?

- A. Increased empagliflozin to 25 mg daily
-  B. Initiate a thiazide diuretic to increase diuresis
-  C. Stop metoprolol
- D. Initiate Imdur 30 mg daily

Commentary

References

Testing Point: The Board-certified AHFTC specialist should Identify beneficial and nonbeneficial treatments for HFpEF.

Correct Answer: C. Stop metoprolol

Rationale: The patient is on very high dose beta-blockers with no indication for the use. In general, it is recommended to avoid beta-blockers in HFpEF due to the negative chronotropic effects and tolerability. However, it is common to see HFpEF patients prescribed beta-blockers likely due to confusion with the appropriate regimen for HFrEF and the common comorbidity of hypertension.

Answer A: There are no data in HFpEF patients to increase empagliflozin above the 10 mg dose.

Answer B: The patient has no evidence of fluid overload and is not hypertensive, so initiating thiazide diuretic is incorrect.

Answer D: There is no benefit to nitrates and it is a class IIIb indication by the ACC/AHA guidelines

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